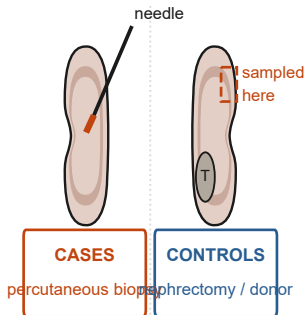


(a) how the tissue was obtained



kidney in situ and percutaneous biopsy already removed;
a 1-2 mm core is taken from the normal-appearing cortex is
and fixed at the bedside from the tumour (T)

the two routes differ in how long the tissue is without circulation and
how it is handled before fixation. Immediate-early genes (FOS, JUN,
EGFR1 and the rest of the 19-gene module) respond to exactly that.

very public DKD cohort, cases are biopsies and controls are not.

The module separates the two routes in 8 of 9 kidney datasets.

(b) what the comparison actually contrasts

conventional design



disease AND handling differ

immediate-early module, Hedges' $g = -0.84$

this paper



only disease differs

immediate-early module, Hedges' $g = +0.08$

Replacing the control with a biopsy of a different kidney disease removes
the handling difference, and the module effect goes with it. What survives
that substitution is the part of the signal that is about the disease.